PEDIATRIC INTENSIVE CARE UNIT

Policy Name : Admission and Discharge criteria

Policy Number : AADC – PICU 1/3 (9)

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PEDIATRIC INTENSIVE CARE UNIT

PURPOSE:

1. To give appropriate monitoring and care for children who are potentially having severe physiological instability requiring technical and artificial support.
2. To have well known criteria for all staff regarding admission and discharge of patients.

SCOPE:

First step: Patient accepted by the Consultant in-charge or Registrar on-call
Last step: Patient assessed by the Staff and Charge Nurse.

ABBREVIATION:

PICU - Pediatric Intensive Care Unit

MRP - Most Responsible Person

DEFINITIONS:

Admission and discharge criteria for PICU provide guidelines and protocol of care, to be initiated when the patient management is accepted or discharge from unit.

MATERIAL / EQUIPMENT:

1. AGFA System
2. OASIS System
3. BMAP System

POLICY STATEMENT:

1. Admission to PICU requires a written order by the admitting physician.
2. The inside Admission will be informed directly by the ID NUMBER or Specialist after duty hours.
3. Referred case from outside hospital, the referring physician has to coordinate with the on-call consultant or specialist for the acceptance.
4. All patient who need PICU admission must be aged between 31 days to 14 years old.
5. All admission must have an initial blood sugar level immediately on admission.
6. Any patient who needs intensive monitoring should be admitted.
7. Trauma cases must be accepted by the consultant on call first before admission.

8. Patients are prioritized by level of nursing care and acuity, need for intensive medical therapy and type of illness.
9. Strictly no watcher allowed.
10. Resuscitation equipment must be ready at all times.
11. Observation of standard precaution must be maintained.
12. All admitted patients are to be fully assessed by the charge nurse and assigned nurse
13. Comprehensive endorsement using SBAR methodology must be done for all newly admitted patients.
14. New patients must be entred on BMAP and OASIS system and discharge on time.

1. Admission criteria:

As stated in Policy AC-PICU 10/3 (1)

1.1. **Cardiovascular** – Arrhythmias, Congenital Heart Failure, Hypertensive crisis, Cardiogenic shock.

1.2. **Respiratory** – Status Asthmaticus, Pneumonia or Bronchiolitis with respiratory failure. Respiratory failure needing ventilation, pneumothorax, moderate and severe croup shall be admitted for at least 6-12 hours before the patient is transferred to the medical for further treatment. All cases suspected and proven to be epiglottis must be admitted until there are no likely chances of developing respiratory obstruction.

1.3. **Endocrine Crisis** – Diabetic Ketoacidosis, Adrenal Insufficiency.

1.4. **Fluid and Electrolytes imbalance** – moderately and severe dehydration due to any cause. Severe electrolytes acid-base imbalance.

1.5. **Hematological** – leukemia / malignancy, agranulocytosis / pancytopenia / aplastic crisis that needing intensive care. Patients of sickle cell anemia with acute chest syndrome or having other complications needing exchange transfusion. Any case presenting with moderate or severe active bleeding due to medical cause.

1.6. **Toxicology** – all patients who have ingested / suspected to ingest poison and need close monitoring should be admitted for at least 12-24 hours as per the discretion of the admitting physician.

1.7. **Metabolic** – Inborn Error Metabolism needing intensive monitoring, Diabetic Ketoacidosis, Symptomatic hypoglycemia or hypocalcemia.

1.8. **Abdominal** – Liver cell dysfunction, Hepatic encephalopathy, Reyes syndrome.

1.9. **Shock** – Anaphylactic, Hypovolemic, Septic and Anemic Shock.

1.10. **Miscellaneous** – all stings should be admitted for at least 12 – 24 hours as per discretion of the attending physician

Objective Parameters Model for admission

Vital Signs

1. Pulse < 40 or > 200 beats/minute
2. Systolic arterial pressure 20 mm Hg below the patient's usual pressure
3. Mean arterial pressure < 50 mm Hg
4. Diastolic arterial pressure > 120 mm Hg *OR above the 95% for age.
5. *Oxygen saturation less than 80% in spite of adequate oxygen supplementation.
6. Respiratory rate > 60 breaths/minute

Laboratory Values (newly discovered)

1. Serum sodium < 110 mEq/L or > 160 mEg/L *

2. Serum potassium < 2.5 mEg/L or > 6.0 mEg/L

3. Haemoglobin less than 5g/dl

4. pH < 7.1 or > 7.7 * Pa02 < 50 mm Hg

5. Serum glucose >500mmol /1 *

6. Serum calcium less than 1.5mmol/l

7. Toxic level of drug or other chemical substance in a hemodynamically or neurologically compromised patient

Radiography/Ultrasonography/Tomography (newly discovered)

Cerebral vascular hemorrhage, contusion or subarachnoid hemorrhage with altered mental

status or focal neurological signs

Electrocardiogram

Myocardial infarction with complex arrhythmias, hemodynamic instability or congestive

heart failure

Sustained ventricular tachycardia or ventricular fibrillation

Complete heart block with hemodynamic instability

2. Discharge criteria:

2.1. Patients admitted to PICU can be transfer to the ward after stabilization and discussion with the primary physician through SBAR methodology.

2.2. Patients who are vitally stable, needing minimal oxygen, suction and nebulization.

2.3. Need minimal monitoring of vital signs / investigations and no longer require inotropic support can be transfer to ward.

2.4. The charge nurse inform directly to the charge of transferring unit through telephone for assign staff according to the acuity.

2.5. The bedside nurse assigned to the patient to be transferred and endorse the patient by using SBAR form and to be counter signed by the receiving nurse and physician.

DOCUMENTATION:

1. Admission and discharge log book
2. Kardex (FM 018)
3. PICU flow chart (FM 052)
4. SBAR Form (FM 88)

RESPONSIBILITIES:

In-charge PICU Consultant will be over all in-charge of the unit both in administration and medical care of the patients including emergency therapy like intubation ventilation, shock therapy and code blue resuscitation.

Head nurse is responsible to orient all staff about the policy
is responsible to make sure that this policy is being followed.

Charge nurses are responsible to accept cases, assist physician and supervise bedside nurses to all the procedure.

Staff nurses are responsible not only for taking care of newly admitted patients but also for arranging things for the discharge of patients.

MEASUREMENT:

1. 100% of PICU admission is according to criteria that are listed.
2. No admission will be accepted unless PICU is informing prior to admission.

REFERENCES:

Ministry of Health, Policy and Procedure Manual, KSA

CROSS REFERENCES: None

DISTRIBUTION:

1. Nursing Administration
2. Quality Department
3. Office of the Executive Director General

REVIEW SUMMARY:

1. Addition on Policy statement